

18 Therapeutic Continuing Bond Interventions with Bereaved Companion Animal Owners

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Introduction

The death of a companion animal (CA)¹ can be a heartbreaking experience, similar to the death of a human. In our clinical practice, we hear bereaved CA owners say things like, “I’m in such pain because of the loss that I can hardly function.” “The loss of Ringo has been the hardest experience of my life,” or, in regard to veterinary euthanasia, “I feel so terrible and hate myself because I couldn’t do anything for them.” Continuing bonds (CB) (i.e., dreams, fond memories, rituals, memorials) can be an adaptive means of coping and grief processing following the loss of a beloved attachment relationship. In this chapter, we examine how CB can be a source of positive coping after the death of a CA and a means for bereaved owners to stay connected to their CA in a healthy way. This will include a discussion about the human–animal bond and grief, disenfranchised grief, social constraints, and CB. We will then discuss clinical intervention strategies, as well as provide pet bereavement vignettes.

Human–Companion Animal Bond and Grief

Many authors have written about the strength and meaning of human–CA relationships (e.g., Carmack, 2003; Toray, 2004). The depth of responses to CA loss is often based on the strength and longevity of relationships individuals had with their pets. In the case of CA loss, many people may not perceive the death of a CA as a cause for intense grief, yet research has clearly demonstrated strong ties between pets and humans and profound reactions to loss (Archer & Winchester, 1994; Carmack, 2003). CAs are described as having the ability to elicit an emotional response from humans. They are described as family members by a large percentage of CA owners (Cowles, 1985; Toray, 2004) and even as a surrogate child (Vevver, 2008). Moreover, humans tend to interpret CAs as having unconditional positive regard for them (Archer, 1997). Recent studies have shown that when humans interact with their dogs, their oxytocin levels increase, mimicking the bonding that occurs between mother and baby (Nagasawa, Mogi, & Kikusui, 2009).

People become emotionally close to their CAs, accept responsibility for them, and grieve deeply when their CA dies (Field, Orsini, Gavish, & Packman, 2009). Research has consistently demonstrated that the grief following the death of a CA can equal or exceed the loss of a beloved human (Field, et al., 2009; Gerwolls & Labott, 1994; Habarth et al., 2017; Packman, Field, Carmack, & Ronan, 2011). Carmack (2003) described several features of CA owner grief: anger, often directed towards the veterinarian; difficulties with concentration; and avoidance of painful reminders. Weissman (1990–1991) noted that the extent of grief in bereaved CA owners may approach clinical proportions, especially in individuals who “valued their pets more than friends or relatives” (p. 246).

Indeed, the grief following the death of a CA manifests itself in similar ways to that experienced after human loss in terms of sleep disruption, and social and psychological challenges (Archer & Winchester, 1994; Habarth et al., 2017; Packman et al., 2011). In one study, Gerwolls and Labott (1994) assessed whether the loss of a CA was different from the loss of a human companion (parent, child, or spouse). At two and eight weeks post-loss, the grief score (Grief Experiences Inventory) of those who had lost a CA were similar to those who had lost a human companion. Importantly, at six months post-loss, there were no statistically significant differences in grief scores between the two groups. In our more recent CA loss research, we have also found scores well above the mean on a measure of complicated grief (ICG; Prigerson & Jacobs, 2001), comparable to human loss (Packman, Field, Carmack, & Ronen, 2011). However, in the case of CA loss, many people may not perceive the death of a CA as a cause for intense grief. Yet, research has clearly demonstrated strong ties between CAs and humans and profound reactions to the loss (Archer & Winchester, 1994; Carmack, 2003).

Disenfranchised Grief and Social Constraints

Disenfranchised grief occurs when a death is not validated or acknowledged by our social connections. Thus, when a person experiences a grief reaction, yet there is no social recognition or validation that the person has a right to grieve or a claim for social support (Doka, 2008), their experience is disenfranchised. Doka (2008) has proposed several categories of disenfranchised grief. Certain circumstances surrounding the death may disenfranchise grief or the ways a person grieves may not be recognized (Doka, 2008). This perspective has been described in relation to CA loss (Packman, et al., 2011; Packman, Carmack, & Ronen, 2012). Packman, et al., (2012) write that the death of a CA may not be fully recognized or validated as a significant loss, resulting in griever's feeling isolated and lacking in support. Statements such as “You can always get another one” or “it’s just an animal” add to bereaved CA owners’ experiences of isolation and distress (Toray, 2004). In fact, encouraging pet owners to get

a new CA soon after a death is commonplace (Podrazik, Shackford, Becker, & Heckert, 2000) and, in most cases, inappropriate.

Disenfranchised grief has been identified as a complicating factor in bereavement (Rando, 1993). Disenfranchised grief usually precludes social support – thus, although the griever has experienced a profound loss, the “grief may have to remain private” (Doka, 2008, p. 235). The bereaved may not be entitled to time off from work nor receive sympathy from others, and therapeutic rituals may be unavailable (Doka, 2008). Neimeyer and Jordan (2002) relate the concept of disenfranchised grief within a broader pattern of empathic failure. They describe such a failure as one that “subtly or obviously invalidates the bereaved person, family, or community’s distinctive narrative of the loss” (p. 95). Empathic failure frequently occurs within the social support systems of those grieving the death of a CA. Empathic failure, or “the failure of one part of a system to understand the meaning and experience of another” (Neimeyer & Jordan, 2002, p. 96), exists in which people do not understand the depth of the human–CA relationship. Their social system also fails to understand the meaning and nature of the human–pet relationship which is typically based on unconditional love, continuing presence, support and consistent companionship.

The authors have observed that social constraints are a key component of disenfranchisement within the CA loss experience (Habarth et al., 2017; Packman et al., 2011), even though the frequency of CA deaths is high (Hewson, 2014). Juth, Smyth, Carey, and Lepore (2015) indicate that social constraints reflect the perceived ability of those in one’s social network to provide support, and not solely the number of contacts in one’s network. To our knowledge, Packman et al., 2011 and Habarth et al., 2017 were the first to explore social constraints in a CA loss sample. Their findings suggest that social constraints are associated with negative psychosocial outcomes such as depression, anxiety, somatization and functional impairment. In addition, it appears that self-compassion buffers the effects of perceived lack of support on depression symptoms.

The Concept of Continuing Bonds with Bereaved CA Owners

The bereavement literature has recently focused on the function of a “continuing bond” in relation to coping (Field & Friedrichs, 2004; Field, Gao, & Paderna, 2005; Stroebe, Gergen, Gergen & Stroebe, 1992) and adaptation following the death of a loved one (Klass, Silverman, & Nickman, 1996). Studies suggest that an emotional connection can be sustained, even though death brings about permanent physical separation (Field, Nichols, Holen, & Horowitz, 1999). According to Field (2008), the resolution of grief involves “a reorganization of the relationship with the deceased” (p. 115) while acknowledging that the loved one has died. The

CB theory (Klass, et al., 1996) is in contrast to the Freudian notion that resolution of grief involves a detachment or termination of the relationship with the deceased (Freud, 1917/1957). Thus, resolving grief does not necessarily involve ending a relationship, but instead involves a restructuring which can continue throughout a lifetime (Field, 2008).

The use of CB theory regarding CA loss was not reported in the professional literature until 2009. Field, Orsini, Gavish, and Packman (2009) were the first to investigate the role of CB in CA loss. They identified four CB expressions (fond memories, sharing of those memories, legacy of the deceased, and use of photographs of the deceased), all of which underscored that an attachment to a CA continues after the death, in a similar way to that found in human loss. In a later CA loss study, Packman et al. (2011) examined and quantified 12 types of CB expressions that occur among bereaved CA owners:

- A continuing connection (sense of deceased CA's presence)
- Belongings (use of deceased pet's belongings)
- Associated places (drawn to places associated with deceased CA)
- Focus on fond memories
- Dreams involving CA
- Thoughts of being reunited with CA
- Living up to ideals or wishes of CA
- Making everyday decisions based on CA's preferences
- Reminiscing with others about CA
- Memorials and rituals (Organizing special events to commemorate CA)
- Intrusive thoughts (mistaking other sounds or sights for CAs)
- Lessons learned from CA (positive lasting influence of CA)

Participants reported a tendency to experience CB as more comforting than distressing (Packman et al., 2011). The extent to which they endorsed each of the CB expressions was comparable to that found in a spousal loss sample (Field & Frederichs, 2004). The results of the pilot study confirmed that individuals experienced dimensions of CB following the death of their CA and emphasized that the majority of bereaved pet owners maintain ongoing, meaningful ties with their deceased CA.

Building upon that research, Packman, Carmack, and Ronen (2011–2012) used qualitative methodology to evaluate specific connections that the bereaved maintained with their CA. Findings complimented the quantitative research findings (Packman, et al., 2011). Participants stated that they were often reticent to share their CB experiences with others, as they did not feel as though they would be validated. Drawing from the same pilot study, Packman et al. (2017) examined the relationship among projective drawings and psychosocial adjustment following pet loss. Drawings were developed and combined with a clinical interview and standardized questionnaires of mental health symptoms. The authors

found that utilizing projective drawings helped the bereaved create and broaden their stories, especially regarding examples of their memories, dreams, or experiences with their CAs that they might not have expressed because of feelings of disenfranchisement.

More recently, the authors have published several larger empirical studies regarding CB and CA loss (Bussolari et al., 2017; Habarth, et al., 2017; Packman et al., 2014; Packman, Bussolari, Katz, Carmack, & Field, 2016). Packman, et al., (2014) qualitatively investigated grief reactions within a cross-cultural sample (United States and French Canada) of bereaved CA owners. Four major themes emerged from the data: lack of validation and support; intensity of the grief and loss experience; strength of the human–CA attachment bond; and CB to cope with loss. Results supported the adaptiveness of CB as well as both its comforting and distressing aspects, similar to previous research (Packman, et al., 2011). Packman, et al. (2016) later qualitatively investigated post-traumatic growth experienced (PTG) by bereaved CA owners. In regard to CB expressions in relation to PTG, findings suggested that some bereaved CA owners are able to transform the meaning of their loss and use CB as a way of coping. Bussolari, et al. (2017) subsequently conducted a qualitative cross-cultural study assessing PTG responses (United States, French Canada, Japan, and Hong Kong) following the death of a CA. Similar to Packman et al. (2016), participants' responses were coded for CB in the context of PTG. It seemed clear that people utilize CB across cultures. In our view, whether or not a CB is adaptive may depend, in part, on whether it is accompanied by constructive meaning following the loss (Neimeyer, Baldwin, & Gillies, 2006). Consistent with this perspective, positive integration of the loss can bring about post-traumatic growth – transformative positive changes (Calhoun, Tedeschi, Cann, & Hanks, 2010) such as finding new possibilities, increased closeness with others and appreciation of life.

In the largest quantitative CA loss study to-date (N=4,336), Habarth, et al. (2017) investigated how CB related to psychosocial functioning and perceived social constraints. Consistent with our earlier studies on CA loss (Packman, et al., 2011) findings indicated that CB were endorsed by over half of the sample. On average, this study found that owners reported experiencing more comfort than distress across CB efforts. We also found that both higher grief and CA attachment scores were associated with using a greater variety of CB expressions (Habarth et al., 2017). We surmised that if someone were more attached to their CA and engaged in a wide variety of activities with them during their lives that they might find it easier to engage in a greater diversity of behaviors to nurture these connections after their CA died. In addition, higher CA attachment and the experience of PTG ratings together related to higher CB comfort scores. In other words, if a person finds meaning and value in the face of recent loss, it might follow that they would find comfort in their efforts to stay connected to their CA.

Clinical Interventions and Continuing Bonds

Empathic Bridging

In order for bereaved CA owners to disclose their emotions and the depth of their loss, a safe place where empathic bridging occurs must be provided. If unconditional positive regard is the best possible condition for therapy in general, then empathic bridging takes that concept one step further by supporting clients with disenfranchised grief experiences in self-expression. Individuals do indeed engage in CB following the death of their CAs and acknowledging this is one of the initial steps in addressing the death of a CA. This is confirmed by our research as well as previous professional writings, in addition to verification from clinical experience. Assessing a client's experience is an essential first step in determining the degree of value of their CB experiences (Packman et al., 2014). For example, a counselor might say to a client:

We're finding that often when one loses a loved animal, that person feels or senses his CA's presence or continues to use the CA's belongings or something similar to try to keep and feel an ongoing connection. I wonder if any of that is happening for you.

Or, one could simply ask, "Do you still feel his or her presence?"

We have found that those grieving for their CAs are reluctant to bring these experiences up for fear that the helping person will think they are "crazy." Yet, when a counselor empathically prompts this area of discussion, clients are often relieved to talk about it. If a therapist normalizes and legitimizes an individual's experience, they become more willing to disclose the experiences. Importantly, just as grief continues to evolve, so too is the experience of one's ongoing CB. Clinicians should acknowledge the evolving nature of CB expressions with their potential for comfort and/or distress. For example, we usually say to our clients:

At times you may be able to look at pictures and feel comfort and at times doing so might feel distressing. Only continue to engage in CB expressions that feel comforting. If something is distressing right now, it may feel comforting in the future.

This is only one example of initiating a conversation to describe the potential comfort one might get from fostering continuing bonds with a CA. It is important for clinicians to help their clients individualize CB expressions based upon each particular person's needs, current intensity of grief, availability of additional supports, and general level of functionality.

Specifically, for some clients, CB may develop immediately after the death, while for others, they may develop over time. At the same time, it is

important that clinicians do not burden bereaved CA owners with expectations that they should develop CB. Such bonds are unique to each person and may develop and change over time. Importantly, it is not whether a person experiences CB that is most critical, but rather the degree of comfort or distress that is experienced from a particular kind of connection. Clinicians might consider exploring with their clients their willingness or resistance to engage in CB expressions. For those who fear they may lose the relationship or lose the feelings for their animal, a discussion about how relationships can, in fact, continue through CB might be appropriate. Again, however, it is most essential that the client understand the fluid nature of these expressions and how they ebb and flow over the course of their lives. Thus, they should be encouraged to engage in a variety of CB to determine, at any given time, which feel most helpful.

For many, the concept of a Rainbow Bridge is helpful and comforting. The Rainbow Bridge describes a person being reunited physically with a beloved CA at the time of the person's death. In our studies, several participants have mentioned the Rainbow Bridge when asked about being reunited with their beloved pets and derived considerable comfort from this image. Conversely, for those clients who have been taught, they will never see their animal again, the finality of their loss can be overwhelming because their religious/spiritual teachings do not acknowledge an after-life. While the CB of Reuniting with a CA may cause distress, clinicians can help clients derive comfort from other CBs.

Finally, those grieving the death of a CA often feel invalidated and unsupported because of their loss. They are reluctant to speak of the intensity of their grief or any experiences of CB either because of previous rejection or fear of their grief being disenfranchised. Statements such as, "It was just a dog" or "What's the big deal, there are lots of cats that need homes" – comments often directed to those in grief – reinforce feelings of disenfranchisement, invalidation, and social constraints (Habarth, et al., 2017). Unfortunately, "Society generally does not acknowledge pet loss as a significant loss, which alienates those who grieve. Clinicians must not fall into the same category" (Podrazik et al., 2000, p. 377). Encouraging clients to seek out supportive situations (e.g., pet loss support groups, online chat rooms) can help them learn what other bereaved individuals do to manage their grief and learn to share their own feelings and coping strategies.

Projective Drawings

Drawings can be a valuable modality for people who have experienced trauma and loss (Reynolds & Prior, 2006). Irwin (1991) suggested that in order for true grief processing to take place, an individual must become fully connected to the emotional challenges of facing loss. Projective drawings are a safe way to access feelings and are useful in allowing the

individual to experience cathartic relief, verbally reflect on their drawing to a non-judgmental listener, provide connectors to the deceased, and analyze the drawing's symbolic meanings (Irwin, 1991).

Packman et al. (2017) examined the relationship among projective drawings and mental health symptomatology for 33 adults grieving the loss of a pet. Results showed that those with greater severity of grief displayed poorer psychosocial adjustment scores on the projective drawings. Projective drawings added clinical utility by introducing a nonverbal form of expression helpful in understanding the severity of grief and in opening up shared communication with those experiencing loss. In terms of CB connections, we suggest two directives (Long, 2005) and recommend using a set of colored markers: (1) Draw the current image you “see” when you are thinking about your CA, that is how do you visualize your animal currently? This first drawing can be used as a mental representation of the participant's relationship with the deceased CA, and (2) draw a picture representing how you are now using the time you would have spent with your CA, that is how have you changed the ways in which you use your energy and time since your CA's death? This directive reflects a future orientation, that is, the extent to which the client has integrated the loss (Long, 2005).

Instead of solely keeping to descriptive characteristics of the drawings, the client can create a fuller explanation and story of the drawings with examples of their memories, dreams, or experiences with their pets. This part of the therapy session can open an avenue for the therapist to form an empathic connection, by listening supportively and validating the experiences, which should hopefully decrease experiences of disenfranchised grief. By supporting their clients who are socially constrained and find it difficult to trust that others will validate their loss, therapists are engaging in empathic bridging. These qualitative findings are similar to Irwin's (1991) statements that making art is useful in allowing individuals to verbally reflect on their drawings to a non-judgmental listener, thereby allowing the analysis of the symbolic meanings behind the drawings. “The focus is on the individual's unique understanding and construction of the loss” (Lister, Pushkar, & Connolly, 2008, p. 250).

Self-Compassion

Self-compassion, as described by Neff (2003a, 2003b) is self-directed attention and care. Inspired by a philosophy of Buddhism, self-compassion includes three components. First, *self-kindness* describes when we are caring and considerate towards ourselves without a sense of judgment. Second, is a *sense of a common humanity*, which refers to the idea that all humans make mistakes and have difficulties. The last tenet refers to the *cultivation of mindfulness* which essentially describes being present in the moment with non-judgmental awareness. Overall, self-compassion can

be described as a healthy attitude towards ourselves when we are struggling. In the psychological literature, self-compassion is related to greater emotional well-being and lower stress (Bluth et al., 2016), plays a role as a predictor for depression and anxiety (Muris, Meesters, Pierik, & de Kock 2016; Ozyesil, & Akbag, 2013), and relates to higher post-traumatic growth (Wong, & Yeung, 2017) as well as higher well-being and less distress (Fong, & Loi, 2016).

At present, there is little research regarding self-compassion and grief (Lenferink, Eisma, de Keijser, & Boelen, 2017). In the only CA loss study, to our knowledge, using the construct of self-compassion, Bussolari, Habarth, Phillips, Katz, and Packman (2018) assessed the use of CB coping strategies in relation to grief and self-compassion. The authors found that the frequency of CB efforts related positively to participants' scores on self-compassion. In essence, we posited that bereaved CA owners with higher self-compassion may be more willing to take risks and engage in more CB expressions to manage grief. This is an important finding, because as clinicians, we want to encourage bereaved CA owners to engage in CB. The more frequently CB expressions are utilized, the better the chance of finding those that are comforting.

For many, being self-compassionate is not easy. This is especially true after CA loss. In our clinical practice, we see bereaved pet owners as some of our most grief challenged. CA owners' feelings of responsibility towards their pet can result in strong feelings of guilt, as well as self-deprecation (Bussolari, et al., 2018). A very important aspect of self-compassion is that it is a skill that can be practiced, learned and assimilated into our daily behaviors. Self-compassion during CA loss can include learning to speak kindly to ourselves, identifying our needs and responding to them, setting boundaries, and/or choosing to only share our grief with those who we know will be supportive. It is important for clinicians to first assess how well a client accepts validation. Often, those who deflect unconditional positive regard from others will have difficulties providing it to themselves. The following interventions are based, in part, upon Germer and Neff's (2013) Mindful Self-Compassion training.

To start, it is important to gain awareness of negative self-talk. A simple way to get clients to pay attention is to have them carry rubber bands. Whenever they notice themselves thinking or saying something negative about themselves, they put a rubber band on their left wrist. When they say or think or do something that is helpful, kind or compassionate (e.g., thinking about euthanasia and feeling like they did the right thing, even though it was still really hard or going out with friends and enjoying themselves without putting themselves down), they put a band on their right wrist. Each day the slate is cleared and the bands are taken off, in order to start fresh. The goal is not to eliminate the left wrist bands – but that is not realistic. Rather, we want to always have more on the right. Germer and Neff (2013) note:

While self-compassion helps people deal with life struggles, it's important to remember that self-compassion does not push negative emotions away in an aversive manner. With self-compassion, instead of replacing negative feelings with positive ones, positive emotions are generated by *embracing* the negative ones.

(p. 858)

Next, clients can be helped to practice an exercise where the three components of self-compassion are taught. Whenever emotional stress come up, a person repeats phrases such as “This is a moment of suffering” (mindfulness), “Suffering is part of life” (common humanity), and “May I be kind to myself” (self-kindness).

Practicing mindfulness is essential for increasing self-compassion. Mindfulness has to do with experiencing our painful emotions without avoidance in order to cultivate compassion. In addition, while we are seeing our negative emotions, we are also not overly identifying with them. Being mindful of our emotions is a necessary precursor to self-compassion. Thus, the practice of noticing our grief and not pushing away can be helpful. If we were to practice mindfulness after CA loss, we might be better able to tolerate our painful feelings, and have the impetus to explore a variety of CB expressions. Thus, rather than “avoid” distress, we would embrace it as an important aspect of the grief process. Cultivating self-compassion can also involve using a loving-kindness meditation. This can include repeating the phrases such as “May I be safe” or “May I be kind to myself” in a formal meditation or throughout the day. Garmer and Neff (2013) describe this practice as “compassionate exposure therapy” (p. 863). This specific loving-kindness practice can evolve from these rote statements to a more person-specific compassionate voice with supportive language such as, for example, “I care about myself and I am hurting because my CA died. May I forgive myself. May I have courage.” Overall, teaching and practicing self-compassionate behaviors can help the bereaved fortify CB expressions by tolerating distressing emotions and nourishing a more self-compassionate foundation.

Conclusion

Research and clinical practice highlight that bereaved CA owners want to maintain continuous and meaningful ties with their CA. Clinical practice suggests that the evolving nature of continuing bonds expressions during the course of grief needs additional exploration. We also need to better understand how CB are experienced in various demographic and cultural groups. Our ongoing international cross-cultural research related to CB expressions and CA loss is significant for clinical practice (e.g., Bussolari et al., 2017). However, there is still a dearth of information regarding the multicultural and spiritual experiences of bereaved CA owners, and psychology bereavement curricula should always include information

about CB and CA loss. Although the grief that bereaved CA owners experience is becoming increasingly acknowledged, there is still a long way to go before pet loss is universally thought of as valid and profound. In fact, many bereaved still do not feel comfortable speaking of their ongoing attachment to their deceased CA. Just as grief related to CA loss has been disenfranchised, so has the experience of CB. Clinicians can affirm and normalize the experiences of CB, especially through the use of a combination of empathic bridging, projective drawings, as well as skills to increase self-compassion. As clients increasingly share their experiences following the death of their CAs with us, our understanding of the CB phenomenon will grow. Our hope is that bereaved CA owners will begin to feel that having a continued relationship with their pet is both a natural and increasingly important part of the healthy grieving process.

Case Scenarios

Case Scenario #1:

Kevin is a young man, aged 26, who has been coming to therapy for the past six months. His reason for the initial visit was:

Nothing feels good to me anymore. I don't get the same pleasure out of the things I used to do. I love to cook, but haven't in a long time. I actually don't seem to have an appetite and have lost a lot of weight. I try to put on a good face for my friends and co-workers, but it is a lot of effort.

He completed a thorough intake indicating that his brother died when he was a teenager, however, he only recently disclosed that his 15 year-old German Shepherd, Rudy, died seven months ago. He expressed that he has been trying to "get over it" and does not feel comfortable talking with his friends and family members. At the time of Rudy's death, a co-worker said to him, "You can always get another one." Kevin noted that Rudy was given to him as a gift when he was 11 years old by his deceased brother and the dog lived with him his entire life. As you ask more questions about the loss, Kevin notes that he had gotten rid of all of Rudy's belongings the day after he died and actively tries to avoid thinking about him.

Discussion Questions:

1. What are some of the reasons that Kevin might be avoiding talking about Rudy's death?
2. What are the areas where an empathic failure could happen with Kevin? What shouldn't you say or ask? How can you engage in empathic bridging so that Kevin feels comfortable talking about Rudy? What could you say to ensure that Kevin feels seen, heard and valued?

3. What questions would you ask to assess Kevin's level of social constraints? How might you help support him in this area?
4. What are the directives for projective drawings related to CB? What would you look for? What would you reflect?
5. What do you imagine would be the kind of "loving-kindness" statements that would be helpful for Kevin and why?
6. How would a mindfulness practice support Kevin's ability to try out a variety of CB expressions?

Case Scenario #2:

Twenty-two-year-old Sheila has been coming to therapy for two weeks because her 5-year-old cat, Sprinkles, died suddenly. This is the first death she has ever experienced, as both sets of grandparents are still living. She recently moved across the country for a job and hasn't yet cultivated a strong support network. Sheila lives alone in a studio apartment and has developed, what she describes as "panic attacks" whenever she comes home to her empty house. Of her job in the tech industry, she notes, "I have a lot of responsibility and have to work very long hours. I can barely get through an hour without running in the bathroom and crying." Sheila blames herself for Sprinkles' death because when she realized she might actually be sick (for the past three days, she seemed lethargic with a decreased appetite), Sheila opted to stay late at work and not take her to the vet until the next morning. While a couple of her co-workers have been supportive, she doesn't want to let anyone know what has happened because, as a woman, she worries that she will be viewed as "weak" or "someone who is not really committed to the mission of the company."

Discussion Questions:

1. How would you start the conversation around CB with Sheila? Are there any CBs that you think would be particularly helpful?
2. What are the areas where an empathic failure could happen as a therapist with Sheila? What shouldn't you say or ask? How can you engage in empathic bridging so that Sheila feels comfortable talking about Sprinkles? What could you say to ensure that Sheila feels seen, heard and valued?
3. What questions would you ask to assess Sheila's level of social constraints? How might you help support her in this area?
4. What are the directives for projective drawings related to CB? What would you look for? What would you reflect?
5. What do you imagine would be the kind of "loving-kindness" statements that would be helpful for Sheila and why?

6. How would a mindfulness practice support Sheila's ability to try out a variety of CB expressions?

Note

1. We use the term Companion Animal (CA) rather than a pet. We agree with Hewson's (2014) belief that pet "... does not adequately reflect the current scientific understanding of animals' sentience, cognitive complexity and individuality, or the complexity and modern reality of the relationships between animals and their owners" (p. 104).

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